

BEFORE THE IOWA BOARD OF SOCIAL WORK

	)	DIA NO. 08DPHSW011
IN THE MATTER OF:	)	
	)	
	)	FINDINGS OF FACT,
JODI L. VOGEL	)	CONCLUSIONS OF LAW,
	)	DECISION AND ORDER

On February 29, 2008, the Iowa Board of Social Work (Board) sent Jodi L. Vogel (Applicant) a preliminary notice of intent to deny her application for a license to practice social work at the independent level. The Applicant appealed and an evidentiary hearing was held on July 7, 2008 at 8:30 a.m. in the fifth floor conference room, Lucas State Office Building, Des Moines, Iowa. The following members of the Board presided at the hearing: David Stout, LMSW, Chairperson; Joyce Westphal, LISW; Katinka Keith, LISW; Beth Harms, LISW, and Mary Tasler, public members. Administrative Law Judge Margaret LaMarche assisted the Board in conducting the hearing. The Applicant appeared and was represented by attorney Becky Knutson. Assistant Attorney General Heather Adams represented the state. A certified court reporter recorded the hearing. The hearing was open to the public, pursuant to Iowa Code section 272C.6(1)(2007). Following the hearing, the Board convened in closed executive session to deliberate their decision, pursuant to Iowa Code section 21.5(1)(f)(2007). The administrative law judge was instructed to prepare Findings of Fact, Conclusions of Law, Decision and Order, in accordance with the Board's deliberations.

THE RECORD

The record includes the testimony of the Applicant, the testimony of the Applicant's supervisor, and the following exhibits:

State Exhibit 1:	Iowa Code sections 154C.1, 154C.3
State Exhibit 2:	645 IAC chapter 280.6, 282.1; Frequently Asked Questions
State Exhibit 3:	Application and Accompanying Materials
State Exhibit 4:	Preliminary Notice of Intent To Deny Licensure, 2/29/08
State Exhibit 5:	Appeal Letter, 3/11/08
State Exhibit 6:	Notice of Hearing

Applicant Exhibit 1:	Resume
Applicant Exhibit 2:	AEA 4 Guiding Principles
Applicant Exhibit 3	School Social Worker Job Description
Applicant Exhibit 4:	The Social Work Dictionary-Excerpts
Applicant Exhibit 5:	DSM IV
Applicant Exhibit 6:	Response to Intervention
Applicant Exhibit 7:	Conner's Rating Scale
Applicant Exhibit 8:	Conner's Tests
Applicant Exhibit 9:	Conner's Tests
Applicant Exhibit 10:	BASC-2
Applicant Exhibit 11:	BASC-2 Teacher
Applicant Exhibit 12:	BASC-2 Self Report
Applicant Exhibit 13:	BASC-2 Teacher
Applicant Exhibit 14:	Parent Rating Scales Report
Applicant Exhibit 15:	Vineland II
Applicant Exhibit 16:	Vineland II Teacher
Applicant Exhibit 17:	Vineland II Parent
Applicant Exhibit 18:	Vineland II Survey Interview Form
Applicant Exhibit 19	RASD-2
Applicant Exhibit 20:	IEP-5/3/2005
Applicant Exhibit 21:	Functional Behavior Assessment
Applicant Exhibit 22:	Response to Intervention, 9-11-2007 and related documents
Applicant Exhibit 23:	IEP, 3/28/2008
Applicant Exhibit 24:	Confidential Team Summary Report
Applicant Exhibit 25:	Student Assessment
Applicant Exhibit 26:	Preliminary Notice of Intent to Deny Licensure
Applicant Exhibit 27:	Supervisor's Report-1 st
Applicant Exhibit 28:	Supervisor's Report-2 nd
Applicant Exhibit 29:	Kelly Van Gelder letter, 1-14-2008
Applicant Exhibit 30:	Molly Twohig letter, 3-31-08

ISSUE

Whether the Board correctly denied the application for social work licensure at the independent level because the Applicant had not established that she was practicing at the master's level by providing diagnosis and treatment of mental and emotional disorders or conditions, while under supervision, in her position at the Northwest AEA?

FINDINGS OF FACT

1. The Applicant earned a Bachelor of Social Work Degree from Augustana College in May 1992 and was licensed in Iowa as a bachelor's level social worker on February 11, 1997. On May 13, 2005, the Applicant earned a Master's Degree in Social Work from the University of Iowa. The Applicant was licensed as a master level social worker in the state of Iowa on August 25, 2005.

The Applicant has been employed full-time as a school social worker by the Northwest Area Education Agency (AEA)¹ from August 2004 to the present. On November 21, 2007, the Applicant filed an Application for Social Work Licensure at the independent level and described her most recent social work practice for the AEA as:

Specialized evaluation, instruction & consultative services to meet the unique educational needs of students who are experiencing educational difficulty, within a problem solving multi disciplinary team. Provide counseling, assessment & other therapeutic interventions of educational relevance to students, parents, and families. Assist in the development of IEPs, provide support related services. Use psychometric assessment to evaluate student needs.²

The application included a Supervision Report signed by the Applicant's supervisor, Kelly Van Gelder, LISW. Ms. Van Gelder documented that she directly supervised the Applicant in weekly meetings from August 16, 2005 through October 31, 2007 for a total of 110.25 hours. In addition, Ms. Van Gelder affirmed on the application that the Applicant:

- Was in a position performing psychosocial assessment, diagnosis, and treatment;
- Had satisfactorily completed the goals and objectives that were established at the onset of supervision and fulfilled

¹ Formerly AEA 4.

² At hearing, the Applicant explained that she took this language directly from the written job description for her position. (Applicant Exhibit 3)

the requirements as outlined in the board's administrative rules regarding supervision; and

- Had adhered to the Board's administrative rules, including rules of conduct.

Ms. Van Gelder recommended that the Board allow the Applicant to sit for the ASWB clinical level examination. (Testimony of Applicant; Kelly Van Gelder; State Exhibit 3)

2. On January 4, 2008, the Board Administrator wrote to Ms. Van Gelder to request a new supervision report form with updated dates of supervised practice, beginning after the Applicant was licensed at the master's level on August 25, 2005. The Board Administrator also asked for a copy of the Plan of Supervision that was developed at the onset of the supervised practice and for verification that the Applicant's social work practice had been at the master's level and had included diagnosis and treatment/psychosocial therapy.

Ms. Van Gelder submitted her written response to the Board on January 17, 2008, stating that the Applicant has been working at the master's level under supervision and that her caseload includes counseling, therapy, diagnosing, and treatment planning with progress monitoring of measurable goals. She further identified the diagnostic tools that the Applicant has administered and interpreted, including the Behavior Assessment System for Children second edition, the Conner's Rating Scales, the NICHQ (National Initiative for Children's Healthcare Quality) Vanderbilt Assessment Scale, and the Reynold's Adolescent Depression Scale (RADS).

Ms. Van Gelder attached a single page written Plan for Direct Supervision, dated August 26, 2005, which provided for face-to-face supervision of the Applicant each Wednesday (with Friday as the alternative day) in the following content areas: practice skills, practice management skills, skill required for continuing competence, development of professional identity, and ethical practice. The plan did not include goals and objectives for the clinical work experience nor specific detail for the content areas of the supervised experience. Ms. Van Gelder also submitted a revised description of the specific hours of supervision from August 26, 2005 through November 9, 2007, for a total of 111.25 hours of supervision. (State Exhibit 3; Applicant Exhibits 27-29)

3. On February 29, 2008, the Board issued its Preliminary Notice of Intent To Deny Licensure to the Applicant on the basis that the Applicant was not providing diagnosis and treatment of mental and emotional disorders or conditions, as required, under supervision, in her position at Northwest AEA. (State Exhibit 3; Applicant Exhibit 26) The Applicant filed a Notice of Appeal. (State Exhibit 5)

4. At the hearing, the Applicant supplemented her written application with her own testimony, the testimony of Kelly Van Gelder, and numerous exhibits. Only following review of this additional explanatory information was the Board able to determine that the Applicant's practice as a school social worker for the Northwest AEA was indeed at the master's level and included performing psychosocial assessments and diagnosis, setting treatment goals based on assessment and diagnosis, and providing direct mental health treatment to students, including psychosocial therapy.

Most AEAs refer students to outside agencies or mental health providers for mental health treatment, and their social workers do not provide diagnosis or therapy. However, the Northwest AEA requires its social workers to provide mental health services directly to students. Northwest AEA utilizes a Response To Intervention (RTI) process that includes defining the problem (assessment/ diagnosis), developing a plan/intervention (treatment plan), implementing the plan (treatment), and ongoing progress monitoring to determine if goals are being reached.

A multidisciplinary treatment team consisting of a school psychologist, social worker, occupational therapist, physical therapist, speech and language therapist, consultant, and special education teachers assess every facet of the child and then design, implement and evaluate intervention plans for student's experiencing educational or behavioral problems. The Northwest AEA social workers specifically address students' social, emotional, and behavioral issues in the school setting and are not directly involved in educational issues, except to the extent that the student's social, emotional and behavioral issues are interfering with the student's ability to meet educational goals.

In her position as a social worker for the Northwest AEA, the Applicant initially interviews the teacher, the parents, and the child to obtain a social history. The Applicant uses various assessment/diagnostic tools, including:

- The Conners' Rating Scales, a standard instrument for the assessment of attention-deficit/hyperactivity disorder (ADHD) and related problem behaviors in children and adolescents. The long version of the Conners' Rating Scales correspond to the official ADHD criteria in the Diagnostic and Statistical Manual of Mental Disorders, Fourth Edition (DSM-IV). (Applicant Exhibits 7-9);
- The Behavior Assessment System for Children, Second Edition (BASC-2), which aids in the clinical diagnosis of disorders that are usually first apparent in childhood or adolescence and which assesses a variety of symptoms noted in the Diagnostic and Statistical Manual of Mental Disorders, Fourth Edition, Text Revision (DSM-IV-TR). (Applicant Exhibits 10-13);
- The Vineland Adaptive Behavior Scales, Second Edition (Vineland-II), an individually administered measure of adaptive behavior. The distinct adaptive domains and subdomains measured by the Vineland-II Survey forms correspond to the specifications identified by the American Association on Mental Retardation (AAMR, 2002) and the DSM-IV-TR. (Applicant Exhibits 15-18); and
- The Reynolds Adolescent Depression Scale-2nd Edition (RADS-2), which is designed to assess the severity of symptomology associated with depression, although it does not provide a diagnosis of a specific and definitive depressive disorder. (Applicant Exhibit 19)

The Applicant also consults and utilizes the DSM-IV on a daily, regular, and ongoing basis. When the assessment process is complete, the Applicant arrives at a DSM IV diagnosis, which she enters in her own file for each student and discusses with her supervisor. However, the diagnosis is not listed on a student's Individual Educational Plan (IEP). The only conclusion that appears on the IEP is that the student is an Eligible Individual (EI).

The Applicant crafts a treatment plan and goal(s) for the student with anxiety, depression, or oppositional defiance disorder, which is based on a DSM IV diagnosis. The Applicant

provides direct one-on-one treatment/psychosocial therapy to each student, at least weekly. The Applicant's treatment plans may include cognitive behavioral therapy and/or play therapy for 6-12 weeks. The Applicant continuously evaluates the student's progress toward treatment goals and may modify the treatment plan and or goals if insufficient progress is being made. If a short-term intervention plan is successful, the student will return to general education. If it is not successful, the Applicant will write an IEP for the student that is based on the social, emotional, or behavioral issues that the student needs to work on. The Applicant will refer students to other mental health providers in the community if necessary. The Applicant maintains regular communication with any other mental health provider who is also providing treatment to one of her students.³ (Applicant Exhibits 20-25, 30; Testimony of Applicant; Kelly Van Gelder)

CONCLUSIONS OF LAW

The Iowa legislature mandates licensure for social workers in Iowa and has specified requirements for three levels of permanent licensure: bachelor social worker, master social worker, and independent social worker. Iowa Code sections 154C.2; 154C.3. Requirements for licensure at the independent level include, in part, possession of a master's or doctoral degree in social work, passing an examination, and having engaged in the practice of social work, under supervision, for at least two years as a full-time employee or four thousand hours prior to taking the examination given by the board. Iowa Code section 154C.3(1)(c)(1)-(4)(2007).⁴

Supervision shall be provided by a social worker licensed at least at the level of the social worker being supervised and qualified to practice without supervision, unless the board determines that such supervision is unobtainable or that supervision by another qualified professional is appropriate. The legislature has specifically authorized the board to

³ The Applicant must be continuously aware of her professional obligation to ensure that she is not duplicating mental health services when other providers are also providing treatment.

⁴ Iowa Code 154C.1"c", only requires independent licensure for private practice. The applicant is not in private practice and an independent level license is not required to work for an AEA.

determine "additional standards for supervision." Iowa Code section 154C.3(5) (2007).

Iowa Code section 154C.1(3) provides, in relevant part:

3. ...The practice of social work for each of the categories of social work licensure includes the following:

...

b. Master social workers are qualified to perform the practice of bachelor social workers and provide psychosocial assessment, diagnosis, and treatment, including but not limited to performance of psychosocial histories, problem identification and evaluation of symptoms and behavior, assessment of psychosocial and behavioral strengths and weaknesses, effects of the environment on behavior, psychosocial therapy with individuals, couples, families, and groups, establishment of treatment goals and monitoring progress, differential treatment planning, and interdisciplinary consultation and collaboration.

c. Independent social workers are qualified to perform the practice of master social workers as a private practice.

Pursuant to the authority delegated by Iowa Code section 154C.3(5), the Board has promulgated rules defining supervised professional practice for social work licensure at the independent level. 645 IAC 280.6 provides, in relevant part:

645-280.6(154C) Supervised professional practice for the LISW.

280.6(1) The supervised practice shall:

a. Be the equivalent of two years of full-time post-master's social work degree practice at the master's level performing psychosocial assessment, diagnosis and treatment; ...

...

c. Have at least 110 hours of supervision which shall be equitably distributed throughout a minimum of a two year period;...

...

"Psychosocial therapy" means a specialized, formal interaction between an LMSW or LISW and a client in which a therapeutic relationship is established and maintained to assist the client in overcoming or abating specific emotional, mental, or social problems and achieving specified goals for well-being. Psychosocial therapy is a form of psychotherapy which emphasizes the interface between the client and the client's environment. Therapy is a planned, structured program based on a diagnosis and is directed to accomplish measurable goals and objectives specified in the client's individual treatment plan. 645 IAC 282.1.

The information provided by the supervisor on the "Plan of Supervision" was insufficient to establish master's level practice during the supervision period. In addition, the description of social work practice provided by the Applicant on her application lacked adequate detail. Moreover, the information initially provided was insufficient to persuade the Board that the Applicant was qualified to sit for the clinical examination.

However, through the testimony and exhibits submitted at hearing, the Applicant has now provided sufficient additional information and detail to allow the Board to conclude that the Applicant has been employed full-time for two-years in a position where she engaged in the supervised practice of social work at the master's level, including providing diagnosis, psychosocial assessment, and therapy. Iowa Code section 154C.3(1)"c"(4); 645 IAC 280.6(1)"a." The Board was further persuaded that the Applicant's master's level practice at the Northwest AEA was adequately and properly supervised for at least 110 hours, and those hours were equitably distributed throughout the two-year period. 645 IAC 280.6(1)"c."

DECISION AND ORDER

IT IS THEREFORE ORDERED that the Board's Preliminary Notice of Intent To Deny Licensure is REVERSED. The Applicant, Jodi Vogel, shall be permitted to sit for the ASWB clinical level examination.

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This findings of fact, conclusions of law, decision and order is approved by the board on August 11, 2008.

Any appeal to the district court from a decision in a contested case shall be taken within 30 days from the date of issuance of the decision by the Board. 645 IAC 11.29.